

Anaphylaxis - Male (Adult)

Algorithm 1017 | Adult (18 years and older) | Gender at birth: Male

Definition

- UAT-only recognition and emergency first-aid pathway for suspected anaphylaxis (a severe, potentially life-threatening allergic reaction); not approved for real-patient or production use.
- Adult patient.
- Male.

Initial assessment

1. [YES_NO] Does the person currently have any sudden airway, breathing, circulation, or reduced-responsiveness symptom described in the emergency question below?
2. [YES_NO] Does the person have their prescribed adrenaline auto-injector available now?
3. [OPEN_TEXT] When did the symptoms start, and was there a possible trigger such as food, medicine, or an insect sting?

Triage and redirect questions

1. Redirect → Allergic Reaction or Hives

Is the reaction limited to the skin or sting site without airway, breathing, circulation, or consciousness features?

A localized reaction without anaphylaxis features belongs in the allergy, hives, or insect-sting pathway.

1. Disposition level 100

After a possible allergic trigger, is there sudden swelling of the lips, mouth, tongue or throat; a tight throat, hoarse voice or difficulty swallowing; noisy, fast or difficult breathing or wheeze; blue, grey or very pale colour; cold or clammy skin; sudden severe dizziness, confusion, collapse or fainting; or is an infant or child unusually floppy or unresponsive? Treat these features as suspected anaphylaxis even when there is no rash.

Resuscitation Council UK guidance recognizes anaphylaxis from airway, breathing or circulation compromise, with or without skin or mucosal changes. Hamad Medical Corporation identifies severe allergic reaction as life-threatening and directs callers in Qatar to request an ambulance on 999.

1. Disposition level 70

Only after all emergency features above have been excluded, is there an isolated rash, hives, itching or mild localized swelling while the person remains fully alert and is breathing and swallowing normally?

Isolated skin symptoms without airway, breathing, circulation or responsiveness problems do not by themselves establish anaphylaxis. This UAT draft routes the person for prompt review because symptoms may evolve; the exact Qatar non-emergency destination and timeframe require local clinical-governance approval.

Advice

Suspected anaphylaxis → call Qatar 999

- Call Qatar 999 for an ambulance immediately and follow the emergency operator's instructions. If the person has their prescribed adrenaline auto-injector, help them use it promptly in the outer thigh according to the device instructions; do not delay the 999 call. Keep the person lying flat with legs raised if tolerated. If breathing is difficult, allow them to sit with legs extended; do not let them stand or walk. If symptoms persist 5 minutes after the first auto-injector and a second prescribed device is available, use it. If the person becomes unresponsive and is not breathing normally, start CPR as directed by the 999 operator. Do not drive the person to hospital. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: symptoms persist after the first auto-injector; collapse or loss of responsiveness; not breathing normally.

Isolated allergic symptoms → prompt review

- Keep the person under continuous observation and arrange prompt clinical advice using the Qatar pathway approved for this UAT environment. Follow the person's existing clinician-issued allergy action plan, if available. An antihistamine may help isolated skin symptoms only when it is already appropriate for that person, but it must not delay adrenaline or Qatar 999 if any airway, breathing, circulation, or reduced-responsiveness feature develops.
- Call Back If: swelling of the lips, mouth, tongue or throat; difficulty breathing, wheeze or difficulty swallowing; dizziness, collapse, confusion or unusual floppiness.

First aid

- Call Qatar 999 for an ambulance immediately and follow the emergency operator's instructions. If the person has their prescribed adrenaline auto-injector, help them use it promptly in the outer thigh according to the device instructions; do not delay the 999 call. Keep the person lying flat with legs raised if tolerated. If breathing is difficult, allow them to sit with legs extended; do not let them stand or walk. If symptoms persist 5 minutes after the first auto-injector and a second prescribed device is available, use it. If the person becomes unresponsive and is not breathing normally, start CPR as directed by the 999 operator. Do not drive the person to hospital. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 25 July 2026)
- Resuscitation Council UK, "First Aid Guidelines – Anaphylaxis", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/first-aid-guidelines> (accessed 25 July 2026)
- NHS.UK, "Anaphylaxis", <https://www.nhs.uk/conditions/anaphylaxis/> (page last reviewed 21 June 2023)

Provenance

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